

Olera Pre-CRP R&D, Commercialization, and Execution Plan

Internal working memo · 31 August 2026 to 1 January 2027 · supersedes the separate pre-CRP commercialization memo and week-by-week plan

1. PURPOSE

What this document is for. Between now and 1 January we have to do two things at once: run the company, and rebuild a grant application. This memo says what we are doing, in what order, and what each piece of work is supposed to prove. It is one document because the two jobs are one job. Every commercial result we produce in the next eighteen weeks is also evidence a reviewer will score, and every question a reviewer would ask is a question a customer or an investor would ask first.

Why we are rewriting rather than editing. The August and September drafting and the mock review exposed real weaknesses, not presentation problems. The proposal asked the CRP to fund work that looked like development. It leaned on a subscription revenue model the company had already moved away from. It described a family product with more maturity than it has. And it treated provider revenue as though it were evidence that institutions would pay for CareNavigator, which it is not. This plan is built to retire those weaknesses with facts by January, not to argue them away.

What has changed in the framing. Earlier versions of this plan described four parallel tracks. That was accurate and it was also flat: it did not say why the tracks belong in the same company. They do, and the reason is the subject of Section 2. Olera is not a family navigation product with two side businesses attached. It is a set of products that each improve a different step of the same pathway, which is why they compound.

How to read the rest. Sections 2 and 3 set the model and the five-year arc. Sections 4 through 7 are the four bodies of work, one per section. Section 8 is the scorecard, organised around the eight risks we are retiring rather than around activity. Section 9 is the week-by-week plan. Section 10 states the end state we are underwriting. Throughout, four words are used precisely and are worth watching for: **known** means we have observed it, **hypothesized** means we believe it and have not shown it, **target** means we intend to hit it, and **stretch** means it would be a very good outcome and we are not planning around it.

2. THE ECOSYSTEM, THE PATHWAY, AND WHERE WE SIT ON IT

Start with the older adult, not with the software. An older adult and the family around them sit inside an ecosystem they did not choose: aid programs with eligibility rules, healthcare organizations that discharge them, payers who bear the risk of what happens next, LTSS organizations, care providers, and a caregiver workforce that is smaller than the demand for it. A recognized need does not become established care by being recognized. It has to travel a pathway. Someone has to assess the need, find services and money that fit, plan a course of action, execute the administrative steps, staff and deliver the care, and then keep learning as circumstances change. The pathway is long, it is mostly unsupported, and it is where the failures happen.

Every Olera product improves a step of that pathway. Caregiver Staffing puts workers into the staffing step, which is the step that most often fails outright: an agency that cannot staff a case cannot establish care no matter how well the family navigated. Client Growth acts on the two steps where a provider and a family have to find each other and then convert that contact into a started case. The provider records and relationships underlie the whole pathway, because you cannot plan around capacity you cannot see. And CareNavigator, dashed in Figure 1, is the end-to-end system the first generation is being built toward. Nothing in that figure is a side business. They are different places to stand on one problem.

The provider products serve five purposes at once, which is why they are worth the four months even though they are not the CRP. They generate revenue. They prove we can sell, deliver, and retain. They deepen the provider relationships and record quality that CareNavigator depends on. They put us inside the operational reality of the agencies whose capacity determines whether care gets established. And they give us a second, independent source of field data about where the pathway actually breaks. One activity, five returns.



Figure 1. The ecosystem an older adult sits inside, the pathway a recognized need has to travel before care is actually established, and where each Olera product acts on that pathway today. Every product improves a step of the same pathway. That is what makes them one company rather than three businesses.

The boundary that must not blur. Provider revenue proves that Olera can commercialize. It does not prove that a risk-bearing institution will pay for CareNavigator. Those are different customers, different budgets, different buying processes, and different evidence. We will say so plainly in the application, because a reviewer who spots us conflating them will discount everything else. CareNavigator has to earn its own commercialization case through product maturity, defensible evidence, and direct buyer discovery. Section 6 is that work.

3. THE FIVE-YEAR ARC

Where the CRP sits in a longer sequence. The CRP is not the company's plan. It is one stage of a longer R&D and commercialization sequence, and the application will be stronger for showing that we know which stage it is. The sequence below is what we would run whether or not the grant is funded; the grant changes the speed and the rigor of the middle of it, not the direction.

Period	What Olera builds	What Olera sells	What that period proves
Phase I and IIB through 2026	Assessment, benefit and program discovery, provider discovery, and the platform foundation. Usability and acceptance evidence with caregivers.	Nothing on the family side. Caregiver Staffing sold to providers with company resources.	Families will use it and the technology works. Known.
Pre-CRP Sep 2026 to Jan 2027	First generation of bounded execution capability, plus verification of the workflows that touch a real application or a real provider.	Two provider products in one narrow beachhead, at real prices, to real agencies.	Olera can sell, deliver, and retain. The product is mature enough to enter late-stage validation. Target.
CRP Years 1 to 3	No new product category. Design and quality controls, independent verification, and the instrumentation a controlled study needs.	Provider products continue on company revenue and are not CRP-funded.	That a mature CareNavigator raises verified care establishment against a concurrent comparison. Hypothesized.
Post-CRP Years 4 to 5	Integration work specific to the first institutional customers: eligibility feeds, referral handoffs, reporting.	A paid institutional proof of concept with a risk-bearing organization, then contracts.	That care establishment moves utilization or cost enough for an institution to buy it at scale. Hypothesized.
Beyond Year 5	Whatever the pathway data says is the next binding constraint.	Institutional contracts alongside the provider business.	Nothing yet. This row is direction, not forecast. Stretch.

Table 1. The five-year arc. Read the last column first: each period exists to convert one kind of uncertainty into one kind of fact, and the labels say honestly which are already known and which are still hypotheses.

The consequence for the application. Development belongs before CRP Day 1 and validation belongs inside the CRP. That single line is what the mock review kept circling, and it is the reason Section 5 exists as a separate body of work with its own deadline. If a reviewer can point at an aim and call it product development, the aim is in the wrong place.

4. NEAR-TERM PROVIDER COMMERCIALIZATION

The beachhead is non-medical home care agencies. Narrow on purpose. Assisted living is in scope only where the same pitch and the same delivery transfer without modification. Home health, skilled nursing, and the more heavily regulated segments are out for these four months, not because they are uninteresting but because a longer sales cycle would eat the whole window. The point of a beachhead is to learn quickly, not to address the market.

Olera Pro Staffing. We recruit, screen, onboard, and match pre-health student workers to agencies, and the agency pays per successful hire. This is the more proven of the two. **Known:** more than 900 students have applied, 25 have been placed, four agencies trialed the service and three of them paid, at roughly \$275 per placement, across multiple semesters. **Target:** ten to fifteen recurring provider customers by January. **Stretch:** 25 agencies averaging about three paid hires a month at \$250, which is roughly \$18,750 a month and about \$225,000 annualized. The \$250 figure is deliberately set below what agencies have actually paid.

Olera Pro Client Growth. We help agencies acquire and convert prospective clients, using managed advertising, landing pages, lead qualification and follow-up, review generation, and search visibility. Those are delivery components, not five products. The commercial question is single: do agencies pay us again because the program produced measurable client acquisition? **Target:** ten to fifteen recurring customers with delivery we can measure. This product is less proven than Staffing, and one of the Week 1 decisions is whether to run both or let one lead.

Both stay company-funded. Neither provider product is a CRP aim, and neither should appear in the aims. They appear in the Commercialization Plan as evidence of commercial capability and in the Environment discussion as evidence that the company can sustain itself, which is precisely what that criterion asks of a prior SBIR recipient.

5. CARENAVIGATOR: WHAT EXISTS AND WHAT COMPLETES BEFORE DAY 1

An honest statement of the current product. CareNavigator today is assessment, benefit and program discovery, and provider discovery, on a platform with real usage and a database of more than 72,000 LTSS records. It has usability and acceptance evidence behind it. What it does not yet have is a mature, bounded execution capability: the part that carries a family from a plan to a completed administrative step. The first generation of that capability is being built now, during Phase IIB and with company resources, and it completes before CRP Day 1. We should describe it that way in the application. Overstating it is the fastest way to lose a reviewer who then reads the aims and finds development hiding inside them.

What the Phase IIB study does and does not give us. The Aim 3 study enrolls 200 caregivers of people with Alzheimer's disease and related dementias, at least half reporting a social need, and measures acceptance and caregiver outcomes: a modified technology acceptance measure, medication adherence, caregiving self-efficacy, and positive aspects of caregiving, with cognitive status as a covariate. It runs through the second quarter of 2027. Two consequences follow and both are load-bearing. It does not measure care establishment, so it cannot be presented as preliminary effectiveness evidence for the CRP endpoint. And it does not report before January, so nothing in the application can depend on its results. Any care-establishment signal we have by January will come from real use of the execution capability, and it will be small. We should present it as what it is.

The pre-CRP completion deadline is Week 4. Not the build: the written definition of what must be complete, what will be verified, and what is explicitly out of scope. Everything in Section 6 depends on being able to describe a specific product to a buyer, and everything in the aim architecture depends on knowing where the line between development and validation falls.

6. INSTITUTIONAL-BUYER DEVELOPMENT

Assume, until shown otherwise, that no institution wants this. That is the productive assumption. Risk-bearing organizations are approached constantly by companies with a plausible story about reducing avoidable utilization, and most of those conversations end in polite interest that never becomes a budget line. A buyer who is enthusiastic in a first meeting has told us nothing. A buyer who tells us the specific reason they would not buy has told us something we can act on. We are not collecting encouragement. We are trying to find out what would have to be true.

The fifteen questions. Every interview works toward answers to these. An interview that produces warmth but not answers should be recorded as a failed interview.

1. Who inside your organization owns the problem of members who need long-term services and cannot arrange them?
2. What does that person get measured on, and over what period?
3. What are you doing about it today, and what does that cost you?
4. Which population would you point this at first, and how many people is that?
5. Would you accept verified care establishment as an endpoint, or do you need a utilization or cost outcome before anyone signs?
6. What comparison would you need? Would a concurrent comparison group satisfy you, or do you require randomization?
7. What effect size would be large enough to matter to you, and what would be too small to act on?
8. How long would you need to see it sustained?
9. What data would you have to give us, under what agreement, and who signs it?
10. What data would you need back, in what format, on what cadence?
11. Who else has to approve this, and what has caused similar proposals to die in your organization?
12. How large would a paid proof of concept have to be for the result to be credible internally?
13. What would you pay for the proof of concept itself, and out of whose budget?
14. If the proof of concept worked, what would the contract look like: per member, per completed episode, shared savings, or something else?
15. What would make you say no even with a good result?

What good looks like by January. Minimum: several substantive interviews with true target buyers, written up, with the disagreements recorded as carefully as the agreements. **Target:** convergence across buyers on the endpoint, the comparison, and the evidence threshold, which is what lets us design the CRP aims against a real requirement instead of our own guess. **Stretch:** written conditional interest in a post-CRP proof of concept from more than one organization. We should not plan on the stretch. Convergence alone materially improves both the Research Strategy and the Commercialization Plan.

A note on letters. A generic letter of enthusiasm is worth almost nothing to a reviewer and we should not spend December collecting them. A letter that names an endpoint, a population, and a condition under which the organization would participate is worth a great deal. Ask for the second kind or do not ask.

7. PRIVATE INVESTORS AND THE LONG-TERM THESIS

Investors sit above the operating work, not inside it. They are not a fifth workstream. They are the audience for what the other three produce, and their confidence should be earned against milestones rather than solicited ahead of them. The thesis has seven parts, and each one retires a different category of doubt.

1. **The demographics are not in question.** The population needing long-term services is growing, survival with chronic disease is longer, and the caregiver workforce is not growing to match. This is the one premise nobody argues with.
2. **The pathway, not the app, is the asset.** The durable position is being present at the sequence through which a need becomes established care, because that is where a great deal of downstream spending is initiated and directed.
3. **Olera already holds unusual supply-side assets.** More than 72,000 records, over 700 claimed listings, and direct commercial relationships with agencies. These are slow to build and hard to copy quickly.
4. **The company can commercialize.** By January this is evidenced rather than claimed: real agencies paying real prices for two products, with retention and unit economics.
5. **The technology has an institutional ceiling far above the provider business.** Provider products are a real business. Institutional contracts for care establishment are a much larger one, and the CRP is the mechanism for producing the evidence they require.

6. Non-dilutive capital carries the expensive part. The validation that institutional buyers demand is exactly what the CRP funds, which means private capital is asked to finance commercialization rather than research.

7. Each product makes the next one cheaper. Staffing deepens the provider network; the provider network improves navigation; navigation generates pathway data; pathway data improves both. The compounding in Figure 1 is the reason to own the whole pathway rather than one step of it.

The careful version of the ambition. The long-term opportunity is to become the infrastructure layer for the care-establishment pathway, in roughly the way Zillow became infrastructure for a transaction it does not itself perform. That is a direction we are building toward, not a position we hold. We do not own the pathway today, the market size is not guaranteed, and the comparison is an analogy rather than a forecast. Saying so is not modesty. An investor who catches an unearned claim discounts the earned ones with it.

8. THE JANUARY READINESS SCORECARD

Organised by risk, not by activity. The question is not how much we did. It is which doubts a reviewer or an investor could still hold on 1 January that we could have removed. Eight risks are worth retiring in this window. Each row below names the risk, what would actually retire it, the minimum that lets the application rest on observed traction, and the stretch.

Risk we are retiring	What would retire it	Minimum by 1 January	Stretch
R1. Olera cannot sell anything	Providers paying real money, more than once, for a product delivered by this team with this headcount.	Multiple providers paying for successful hires and several paying for client growth, with delivery documented.	25 agencies averaging about 3 paid hires a month at \$250, roughly \$18,750 a month.
R2. Olera cannot deliver or retain	Repeat purchasing, measured churn, and an acquisition and delivery cost we can state without hedging.	At least one clear repeat purchase and a stated cost per acquired customer.	Retention and margin stable enough to project a run rate.
R3. CareNavigator is not actually built	The first generation of execution capability completed and testable on company and Phase IIB resources, before CRP Day 1.	Core execution product complete and testable, with the critical workflows verified.	Preliminary care-establishment signal from real use, however small.
R4. Nobody knows what a buyer would require	Direct conversations with risk-bearing organizations that answer the fifteen questions in Section 6 the same way.	Several substantive interviews with true target buyers, written up.	Convergence across buyers on endpoint, comparison, and evidence threshold.
R5. There is no market pull	A buyer willing to say what it would take for them to participate in a paid proof of concept.	One buyer engaged enough to review a draft proof-of-concept design.	Written conditional interest from more than one buyer.
R6. The CRP asks for the wrong thing	An aim architecture built only from allowable late-stage activities, with development moved before Day 1.	Revised aims drafted against the buyer requirements and the allowable-activity list.	Reviewers would struggle to name a single development task inside the aims.
R7. The team cannot carry both	Eighteen weeks of selling and drafting running at once without either stopping.	Weekly numbers unbroken from Week 1, and both documents drafted by Week 12.	Week 16 closes with everything complete and the last two weeks unused.
R8. There is no money after the CRP	Investors who understand the milestones and stay engaged as they are hit.	Investors briefed on the milestone sequence and still in the conversation.	Support letters indicating financing interest conditional on CRP and commercial milestones.

Table 2. The January scorecard, organised by the risk each result retires rather than by the activity that produced it. A number that retires no risk does not belong in the application. Minimums are what the application needs to rest on observed traction; stretches are planning targets, not commitments.

These are planning figures, not commitments to NIH. They should be refined with David and TJ in Week 1 against real sales and delivery capacity. The application will report what actually happened.

Scored criterion	What the January evidence puts in front of the reviewer
Significance	Market pull evidenced by buyer interviews rather than asserted, plus a competitive read from agencies that actually bought something (R4, R5).
Investigator(s)	A team that commercialized a product during the drafting period, with revenue and retention to show for it (R1, R2, R7).
Innovation	A first-generation execution capability that exists and can be demonstrated, not described (R3).
Approach	Aims containing only allowable late-stage activities, with the development boundary visible on the timeline (R6).
Environment	A company sustaining itself on revenue while running the project, which is exactly what this criterion asks about a prior SBIR recipient (R1, R2).
Commercialization Plan	Five-year milestones anchored to observed unit economics rather than to a projection, and a revenue stream we have actually collected (R1, R2, R8).
Fundraising Plan	Named investors who have watched the milestones land, engaged against a defined post-CRP financing need (R8).

Table 3. Every January result is aimed at a scored criterion. The mapping is the test of whether a given week of work is worth doing: if a task cannot be traced to a row here, it is not pre-CRP work.

How the mock-review weaknesses map onto this. The development-inside-the-aims criticism is R6. The stale revenue model is R1 and R2, which replace a projection with collected revenue. The overstated family product is R3, which is why Section 5 states the current product honestly and sets a Week 4 deadline for the CRP-entry line. The missing market pull is R4 and R5.

9. WEEK BY WEEK

How to read this section. Eighteen weeks, this week to submission. Each week has one job, a short numbered task list, and a milestone that either happened or did not. Owners are deliberately absent: they are assigned in the Week 1 session, not by this memo. The bracketed tags trace each task to the risk in Table 2 it retires. A task with no traceable risk should not be on the list.

The date that matters is 18 December, not 1 January. Week 16 is the internal deadline. January 1 is a federal holiday, and the two weeks before it are the least reliable of the year. The plan therefore finishes the work before the holidays and treats the last fortnight as buffer and submission mechanics. Four weeks are checkpoints where the plan either holds or gets re-cut. Two are holidays and carry no planned work, because a plan that needs Christmas to succeed has already failed.

Event	When	Why we go	What we must leave with
Nashville Healthcare Sessions	Confirm. Typically autumn	Provider and operator density in the segment we are actually selling to.	Named agency prospects and at least one buyer introduction.
ARC Summit	Confirm. Typically autumn	Aging and care-innovation buyers and the investors who follow them.	Two buyer conversations booked for the following week.
HLTH	Confirm. Typically early Nov	The single highest concentration of risk-bearing buyers and investors in the four months.	Feedback on proof-of-concept design and decision thresholds from at least three organizations.
IHI Forum	Confirm. Typically early Dec	Quality and population-health leaders who own the outcomes we would be contracting against.	A read on which endpoint a quality leader finds credible.
JP Morgan Healthcare	Confirm. Typically early Jan	After submission. It is a constraint on the calendar, not an activity in the plan.	Nothing. Do not let January travel planning consume December.

Table 4. Conferences are worked, not attended. Each row states the one thing that makes the trip worth its cost. **No date here is confirmed.** Confirm all five before approving the week plan, and move the affected weeks if they differ.

The standing rhythm, all eighteen weeks. Monday, thirty minutes on the numbers: outreach, meetings, offers, conversions, active customers, delivered outcomes, repeat purchasing, revenue, churn. Friday, numbers updated and one line on what moved. End of each month, an honest read against Table 2 saying which level we are tracking to. This is what makes the weeks below executable rather than aspirational.

Week 1 August 31 to September 4 **CHECKPOINT**

Decide the shape of the sprint

1. Working session with David and TJ: agree the minimums in Table 2 and whether both provider products run or one leads [R1 R7]
2. Confirm the five conference dates in Table 4 and move any week they break [R4]
3. Stand up the weekly numbers and write down today's baseline [R1 R2]

Milestone. The plan is approved and the baseline exists.

Week 2 September 7–11

Package the two offers

1. Finalize Client Growth and Staffing pricing, terms, and what delivery includes [R1]
2. Build the target list of non-medical home care agencies in the beachhead [R1]
3. Write the sales workflow from first contact to signed [R1 R2]

Milestone. Anyone on the team could run a first sales call unaided.

Week 3 September 14–18

Start selling and start listening

1. Begin concentrated outreach against the target list [R1]
2. Finish the buyer interview guide from the fifteen questions in Section 6 [R4]
3. Book the first risk-bearing-organization conversations [R4 R5]

Milestone. Outreach is live and the first buyer calls are on the calendar.

Week 4 September 21–25

Draw the CRP-entry line

1. Write down exactly what CareNavigator must complete before CRP Day 1, and what it will not do [R3 R6]
2. First two buyer interviews [R4]
3. Keep selling [R1]

Milestone. The development-versus-validation boundary is written, not assumed.

Week 5 September 28 to October 2 **CHECKPOINT**

First close and Month 1 checkpoint

1. Close the first paying provider [R1]
2. Two more buyer interviews [R4]
3. Review against Table 2 and say which level we are tracking to [R7]

Milestone. Revenue exists, or we know by 2 October that it will not.

Week 6 October 5–9

Make delivery repeatable

1. Simplify delivery to what the first customers actually needed [R2]
2. Continue buyer interviews [R4]
3. Begin verification planning for the workflows that touch a real application [R3]

Milestone. The second customer costs less to serve than the first.

Week 7 October 12–16

Second cohort and first repeat

1. Convert a second cohort and look for the first repeat purchase [R1 R2]
2. Continue buyer interviews [R4]
3. Draft the revised aim architecture from what buyers are saying [R6]

Milestone. One customer has bought twice.

Week 8 October 19–23

Requirements converge

1. Write up where buyers agree and where they do not [R4]
2. First investor update, using observed traction only [R8]
3. Continue the execution-capability build [R3]

Milestone. We can state what a buyer would require, in their words.

Week 9 October 26–30 **CHECKPOINT**

Month 2 checkpoint and conference prep

1. Review against Table 2; re-cut the plan if the minimums are out of reach [R7]
2. Book conference meetings with buyers and investors [R4 R5 R8]
3. Complete the critical execution capabilities [R3]

Milestone. Either the January minimums are still live, or the plan changes today.

Week 10 November 2–6

Conference week

1. Buyer and investor meetings against the Table 4 objectives [R4 R5 R8]
2. Get concrete feedback on proof-of-concept design and decision thresholds [R5]
3. Keep the sales cadence running behind the travel [R1]

Milestone. Three organizations have reacted to a specific proof-of-concept design.

Week 11 November 9–13

Convert the conference

1. Follow up every conversation within five days or it did not happen [R5 R8]
2. Increase the paying-provider count [R1]
3. Begin the buyer-informed Research Strategy draft [R6]

Milestone. Every conference conversation has a next step with a date.

Week 12 November 16–20

Draft the application

1. Research Strategy draft on the new aim architecture [R6]
2. Commercialization Plan draft against observed traction [R1 R8]
3. Request support letters [R5 R8]

Milestone. Both documents exist end to end, however rough.

Week 13 November 23–27 **HOLIDAY**

Short week, hold the line

1. Thanksgiving. Keep the sales cadence and chase letters [R1 R8]
2. Start nothing new

Milestone. Nothing slips.

Week 14 November 30 to December 4

Prove the economics

1. Show repeat purchasing and churn; compute acquisition cost, delivery cost, and margin [R2]
2. Consolidate whatever preliminary CareNavigator evidence exists [R3]
3. Full internal read of both drafts [R6 R7]

Milestone. The unit economics are numbers, not estimates.

Week 15 December 7–11

Lock the evidence

1. Freeze the traction figures that go into the application [R1 R2]
2. Identify post-CRP proof-of-concept candidate organizations [R5]
3. Second draft of both documents [R6]

Milestone. No number in the application will move again.

Week 16 December 14–18 **CHECKPOINT**

Everything complete

1. All support letters in hand [R8]
2. All sections complete and internally reviewed [R6 R7]
3. Final scorecard: report actual traction against Table 2, transparently [R1]

Milestone. The application is submittable. This is the real deadline.

Week 17 December 21–25 **HOLIDAY**

Buffer only

1. Holiday week. Buffer for anything that slipped
2. Start nothing new

Milestone. Nothing is required of this week.

Week 18 December 28 to January 1 **CHECKPOINT**

Submit

1. Final compliance check against the SF424 and NOFO requirements [R6]
2. Assemble, proof, and submit [R6]

Milestone. Submitted.

10. THE JANUARY END STATE

What we are underwriting. On 1 January, Olera submits an application in which the commercial claims are observations rather than projections, the aims contain only late-stage validation because the development happened before Day 1, the evidence requirements were written by the institutions that would eventually buy, and the investors who would finance the post-CRP proof of concept have watched the milestones land rather than reading about them. That is the whole objective. Every week in Section 9 exists to move one of the eight risks in Table 2 closer to retired.

The Valley of Death, stated plainly. Olera's remaining gap is not whether families need navigation, and not whether the company can build a provider audience. Both are settled. It is whether a mature CareNavigator can reliably increase verified care establishment, with evidence strong enough that a risk-bearing organization will fund a paid proof of concept and then buy. Before submission we demonstrate commercial execution on our own resources and let real buyers define the evidence CareNavigator must produce. The CRP is then directed narrowly at the late-stage validation that produces it.

What we owe each other by Friday of Week 1. Agreement on the Table 2 minimums against real capacity. A decision on whether both provider products run or one leads. Confirmed conference dates. Named owners for each of the eighteen weeks. And the first row of the weekly numbers, so that every later claim in this plan has a baseline to be measured against.